

SECTION 6

Water

The drinking water advisories that have outlasted the children who were born under them, the industrial effluent that contaminates treaty waters, and the Alberta government's documented opposition to enshrining First Nations' right to clean water in law

In 2010, the United Nations declared access to clean water a human right. In 2026, that right continues to be denied to approximately forty First Nations in Canada that have been under long-term drinking water advisories — some for more than thirty years. The inequality is not incidental. It is the documented record of every federal government and every provincial government that has held the file.”

This section documents the water crisis as it is experienced by Indigenous communities in Canada generally and by Indigenous disabled Albertans specifically. The scope is deliberately broader than drinking water alone. It includes three interlocking dimensions: drinking water advisories on reserve, industrial water contamination in treaty territories (particularly downstream of the Athabasca oil sands), and the legislative framework — or the absence of one — that governs whether a First Nation person in Canada has any enforceable right to safe water at all.

Three findings structure the section. First, that the long-term drinking water advisory system is not a matter of isolated infrastructure failures but a sustained funding and policy failure across multiple federal governments, documented most recently by the Auditor General of Canada in 2025 as “unsatisfactory.” Second, that industrial water contamination — particularly the Athabasca oil sands tailings complex, now exceeding 1.5 trillion litres and growing — is a separate water harm borne disproportionately by downstream First Nations whose treaty rights to the river predate the industrial development by a century. Third, that the Government of Alberta formally opposed Bill C-61, the federal First Nations Clean Water Act, on the stated ground that enforceable First Nations water rights would “undermine competitiveness, and delay project development” — a position that places provincial resource-extraction economics in direct conflict with the human right to water as recognized by the United Nations and the Crown's own treaty obligations.

The connection to disability is direct and documented. Contaminated drinking water produces skin infections, infectious disease, chronic cardiovascular and renal burden, and in the Athabasca watershed specifically, elevated rates of rare cancers. Water scarcity and advisory-

period anxiety produce mental health harm documented by federal Indigenous Services Canada itself. The children currently being born in communities under long-term advisories will enter adulthood with a documented health-outcome profile different from their non-Indigenous peers — a difference that, by the time it is adjudicated by provincial disability benefit programs, will be characterized as the applicant's personal medical history rather than the product of federal policy failure.

Part 1: The long-term drinking water advisories

Current scale

As of the most recent Indigenous Services Canada reporting in early 2026, approximately 40 long-term drinking water advisories remained in effect in 37 First Nations communities across Canada. 152 long-term advisories have been lifted since November 2015. The arithmetic is simple: for every advisory lifted, another has been added. In 2025 alone, 11 new long-term advisories were added while 3 were lifted. In 2024, six were added and four lifted. In 2023, three were added and six lifted. The net progress, over the three-year window immediately prior to the publication of this report, has been approximately zero.

The Council of Canadians documents, in its ongoing tracking, that 73 percent of First Nations water systems operate at high or medium risk of contamination. The Parliamentary Budget Officer identified, prior to 2020, a \$138 million annual shortfall in federal funding for operation and maintenance of on-reserve water treatment equipment. That shortfall has not been publicly reconciled.

The 2025 Office of the Auditor General report on Indigenous Services Canada's water file rated the federal department's progress "unsatisfactory." The report documented that nine of the persisting long-term advisories had been in effect for more than a decade. The oldest persisting advisory, affecting Neskantaga First Nation in Ontario, has now been in place continuously since 1995 — over thirty years. Children born in Neskantaga in 1995 are now thirty-year-old adults who have never drunk tap water from their home community without boiling it first.

The children who were born under the advisory are now the adults who will never be able to teach their own children what safe tap water tasted like, because they never learned themselves. The loss is not only biological. It is cultural. A generation has grown up understanding that the water is a danger, not a relationship."

Why the advisories persist

The official reasons, drawn from Health Canada and the Assembly of First Nations joint analysis, are: disinfection failures (32 percent), equipment failure (30 percent), microbiological quality (18 percent), turbidity (6 percent), source water quality (6 percent), and other operational

issues (8 percent). What this breakdown does not capture is the underlying structural reason: the federal funding model for on-reserve water infrastructure has been inadequate for decades. The \$4.61 billion invested since 2015 on water projects has lifted 147 advisories but left the systemic problem unaddressed, because the funding model has covered capital construction without providing durable funding for operation, maintenance, and operator training. New water treatment plants are built. Without ongoing O&M funding and trained operators, plants fail. New advisories are issued. The cycle repeats.

On January 10, 2026, three severe drinking water emergencies were simultaneously active in First Nations communities: Pimicikamak Cree Nation in Manitoba (downed hydro line rendering the water treatment plant inoperative), Kashechewan First Nation in Ontario (water plant treatment failure), and Kugaaruk in Nunavut (sea water intrusion into the treatment plant). Indigenous Watchdog observed, in its tracking of these three events, the comparative question that the sustained advisory pattern raises: how quickly would equivalent infrastructure failures be addressed in a non-Indigenous southern Ontario or Alberta community of equivalent size?

The 2021 class-action settlement

On December 22, 2021, the Federal Court and the Court of Queen's Bench of Manitoba jointly approved the settlement agreement in the First Nations drinking water class actions. The settlement covered approximately 142,000 individual class members from 258 First Nations, together with 120 Impacted First Nations, for drinking water advisories lasting one year or longer between November 20, 1995 and June 20, 2021. The settlement totaled approximately \$8 billion, of which \$1.8 billion was allocated to direct compensation for individuals and \$6 billion was allocated to water infrastructure projects. A further commitment was made to modernize the legislative framework governing First Nations drinking water.

The settlement is documented, acknowledged, and reflected in Alberta's own administrative practice. The Alberta Income and Employment Supports Training and Health Benefits Policy Manual formally exempts compensation payments received under the First Nation Drinking Water Advisory Class Actions Settlement (2021) as "Harm Done" compensation, meaning the province formally acknowledges that the federal government caused the harm for which the settlement compensates. An AISH or Income Support recipient who receives a settlement payment under this program does not have the payment counted as income for benefit calculation purposes — because the province's own policy infrastructure treats the payment as compensation for harm, not as new income.

This is structurally significant. Alberta's provincial policy has formally incorporated, into its administrative framework, the legal acknowledgment that federal water policy failure constitutes a harm. That same province then, as documented below, opposed the federal legislation that would have prevented the harm from recurring.

Part 2: The legislative vacuum and the killing of Bill C-61

What Bill C-61 was, and what it would have done

The Safe Drinking Water for First Nations Act, first enacted in 2013, was repealed by the federal government in June 2022 — partly in response to sustained First Nations advocacy that the Act had been developed without adequate consultation and had not produced results. Its replacement was announced as forthcoming. Bill C-61, the First Nations Clean Water Act, was first read in the House of Commons on December 11, 2023. The bill was the federal legislative response to the 2021 class-action settlement's requirement for a modernized drinking water legislative framework.

Bill C-61 would have, at minimum, proposed to recognize safe drinking water as a basic human right for all First Nations; established enforceable water quality standards on reserves comparable to those in effect for non-Indigenous communities; set requirements for source-water protection for the watersheds flowing onto First Nations lands; and created a statutory framework through which drinking water advisories could be challenged and compelled to be addressed on timelines that would not stretch into decades. The legislation was supported by the Assembly of First Nations, the Chiefs of Ontario, the British Columbia Assembly of First Nations, and Indigenous organizations across the country.

The bill was sent to the Standing Committee on Indigenous and Northern Affairs in June 2024, held committee meetings through June and September 2024, and was expected to pass in 2025. It did not. Parliament was prorogued in January 2025. Bill C-61 died on the order paper. Prime Minister Mark Carney's minority Liberal government has committed to reintroducing successor legislation, but as of March 2026 no timeline had been announced. As the British Columbia Assembly of First Nations observed on World Water Day 2026: "During Prime Minister Mark Carney's campaign, he made explicit commitments to 'Enshrine First Nations' right to water into law.' History has demonstrated that basic legislation is not enough to ensure First Nations human rights to access safe drinking water are honoured by Canada."

Alberta's documented opposition

On July 10, 2025, the governments of Alberta and Ontario formally placed themselves on record as opposed to Bill C-61. The stated grounds for this opposition, as summarized by Indigenous Watchdog in its tracking of the legislation, were that passing the Act would "undermine competitiveness, and delay project development." In other words: resource extraction projects, in the provincial government's stated position, should take priority over enforceable First Nations rights to safe drinking water.

The position is documented. It is not contested. And it is consistent with the provincial government's broader pattern of resource-extraction-first policy, including its concurrent advocacy, as detailed below, for permitting the oil sands industry to release treated tailings water into the Athabasca River.

The province that opposed enshrining First Nations' right to clean water in federal law on grounds of 'competitiveness' is the same province proposing to release treated oil sands tailings into the river from which downstream First Nations communities draw their drinking water. These are not two separate policies. They are one policy, expressed twice."

Part 3: Industrial water contamination — the Athabasca tailings complex

The scale

The Athabasca oil sands represent the world's fourth-largest oil deposit by reserve size. The extraction process produces large volumes of process-affected water, which is stored in tailings ponds located on industry-operated sites in northern Alberta. As of 2024, the total stored tailings volume across the oil sands was approximately 1.5 trillion litres, held in ponds covering 270 square kilometres — an area larger than the city of Vancouver. The ponds have grown in volume at approximately 42 percent every five years since 2005.

The contents of these ponds include, in quantities established by peer-reviewed analysis and federal monitoring: naphthenic acids (designated among the most persistent and toxic components of oil sands process-affected water, with documented endocrine-disruption and narcosis effects); residual bitumen; polycyclic aromatic hydrocarbons; heavy metals; and a range of organic compounds whose cumulative toxicity to aquatic life and human health has been documented across multiple studies.

Downstream of the tailings complex, along the Athabasca River and into the Peace–Athabasca Delta and Lake Athabasca, live the members of Mikisew Cree First Nation, Athabasca Chipewyan First Nation, Fort McKay First Nation, the Fort Chipewyan Métis, and other communities whose treaty territories predate oil sands development by a century. The communities draw drinking water, harvest fish, hunt, and gather food and medicine from the same watershed that receives the runoff, seepage, and — in documented incidents — direct spill discharge from the tailings complex.

The Kearl disaster

In January 2023, Imperial Oil's Kearl oil sands site in northern Alberta experienced a tailings pond breach. Approximately 5.3 million litres of process-affected water escaped containment. The incident was not publicly disclosed for several days. When public disclosure followed, it included a second finding: a separate drainage system at the same facility had been leaking tailings into groundwater for at least nine months, entering the environment and migrating toward surface water. Indigenous communities downstream of Kearl — most immediately Fort Chipewyan, situated approximately 250 kilometres downriver — were not directly notified by either Imperial Oil or the Alberta Energy Regulator. They learned of the spill through the Alberta Energy Regulator's posting of an environmental protection order.

The community's response was immediate and practical. Melaine Dene, acting director of government and industry relations for Mikisew Cree First Nation, began coordinating emergency bottled water delivery before the winter ice road connecting Fort Chipewyan to the south melted. Chief Allan Adam of Athabasca Chipewyan First Nation, testifying at a federal parliamentary hearing two months after public disclosure, described the experience of turning on a tap in his own home in the days after the spill without knowing whether the water was safe: "I had to turn that tap on to cook my food. I drink two cups of coffee every morning. Where does that water come from? Knowing that turning that tap on could be detrimental to my health..."

In March 2024, Athabasca Chipewyan First Nation filed suit against the Alberta Energy Regulator over its handling of the Kearl incidents and alleged regulatory failures. The case remains before the courts. In April 2023, while the Kearl response was still unfolding, a separate incident at a Suncor settling pond released approximately 6 million additional litres into the Athabasca River.

The documented health consequences

As documented in Section 5 of this report, the April 14, 2026 Mikisew Cree First Nation community health study found approximately 25 percent higher cancer rates in Fort Chipewyan and surrounding communities than in the comparison Alberta population, with 149 cancer cases documented across 24 different cancer types in a community of 641 people. The specific cancer-type distribution is consistent with exposure to the organic compounds documented in oil sands process-affected water and in the environmental media of the Athabasca watershed. Dr. John O'Connor, the physician who first raised public concern about cancer rates in Fort Chipewyan, was professionally investigated for his work — a pattern documented in Section 5 as a case of professional retaliation against whistleblowers who identify environmental harm to Indigenous communities.

The Medscape analysis of First Nations water conditions (September 2024) documents the broader health profile of communities under long-term drinking water advisories: skin infections, higher-than-average cancer rates, and a documented trauma and mental-health burden including suicidality among young community members. Kumar and colleagues at the University of Manitoba have documented, in peer-reviewed analysis of bacterial diversity and the antibiotic-resistance profile of water stored in cisterns in Manitoba First Nations, the specific infectious-disease risk profile produced by the combination of contaminated source water, inadequate treatment, and cistern-based storage.

Alberta's "treat and release" proposal

As of 2025, the Government of Alberta, under Premier Danielle Smith and Environment Minister Rebecca Schulz, is actively pursuing a policy framework that would permit oil sands operators to treat tailings water and release the treated water into the Athabasca River. The proposal is advanced through an industry-dominated provincial committee with a single Indigenous

representative, in parallel to — and in effective competition with — the federal Crown-Indigenous Working Group (CIWG), which includes representatives of nine downstream Indigenous communities and was established in 2021 specifically to evaluate tailings management alternatives that protect Indigenous rights and health.

In October 2025, the Mikisew Cree First Nation, Athabasca Chipewyan First Nation, Keepers of the Water, Indigenous Climate Action, Environmental Defence, the Canadian Parks and Wilderness Society Northern Alberta, the Canadian Association of Physicians for the Environment, and the Council of Canadians (Edmonton Chapter) issued a joint statement rejecting the “treat and release” proposal. The joint statement characterized the Alberta provincial committee’s recommendations as undermining the federal CIWG process, bypassing established Indigenous consultation standards, and putting downstream communities and ecosystems at serious risk through reliance on unproven treatment technology. Chief Allan Adam of Athabasca Chipewyan First Nation commented: “This province works backwards.”

As Jesse Cardinal, executive director of the Athabasca Keepers of the Water, stated in correspondence to the federal Minister of Environment and Climate Change: “Releasing tailings to the watershed is not a safe or just option for solving Alberta’s growing tailings crisis. It is a handout to oil companies that allows them to avoid reclamation costs. It is a decision that [means] northern Indigenous communities and lands will remain a sacrifice zone for the profit of settler governments, southern populations and some of the world’s richest corporations.”

The Fisheries Act currently prohibits the deposit of deleterious substances into water bodies containing fish unless authorized by federal regulation. No such regulation currently exists for oil sands mines. The Alberta provincial proposal, to be operational, would require federal regulatory action to create that authorization. As of the publication of this report, no such authorization has been granted.

Part 4: Water as upstream cause

The direct pathway

The argument developed in Section 4 on housing — that on-reserve housing conditions are themselves upstream causes of the disability profile the AISH program adjudicates — applies with equal force to water. The causal pathways are established and peer-reviewed:

- Contaminated drinking water produces gastrointestinal infection, skin infection, dermatologic conditions, and in populations with compromised immune function, serious systemic illness.
- Boil-water advisory compliance produces chronic logistical and economic burden, sleep disruption in households with infants (formula preparation requires boiled water), and documented mental health and trauma effects across populations that have been under advisory for years or decades.

- Long-term exposure to organic compounds characteristic of oil sands process-affected water — naphthenic acids, polycyclic aromatic hydrocarbons, heavy metals — is associated in the peer-reviewed toxicology literature with endocrine disruption, hepatic injury, carcinogenesis (particularly biliary tract and bile duct cancers documented in the Mikisew Cree First Nation community health study), and cardiovascular effects.
- Water-borne industrial contamination in traditional territory produces secondary harms: loss of traditional food security, loss of cultural and ceremonial practices tied to water, and the community-wide chronic stress documented in the health literature as an independent predictor of mortality.

The AISH and ADAP intersection

An Indigenous disabled Albertan whose disability traces to any of the pathways documented above appears in the AISH system as a person with a qualifying medical condition. The cause of the condition is, from the perspective of the provincial adjudication process, irrelevant. The physician fills out the form; the recipient is approved or denied on the basis of medical severity and permanence; the benefit is paid, or clawed back, or transitioned to ADAP under the July 1, 2026 framework.

What the adjudication process does not record — because it is not designed to — is that the person's condition is frequently the downstream biological expression of federal and provincial policy decisions that this section has documented. The federal government failed to adequately fund water infrastructure on the applicant's reserve, and the applicant developed the condition as a result. The provincial government opposed the federal legislation that would have made the failure remediable in law. The same provincial government now administers the disability benefit the applicant requires and, as documented throughout this report series, administers it in a manner that minimizes federal disability recognition through CDB clawback, strips appeal rights through Bill 12, reduces the benefit rate through the ADAP transition, and reassesses eligibility under a standardized framework that makes no allowance for the Indigenous-specific causal pathway that produced the disability in the first place.

The loop closes. The population inside the loop does not.

What this section establishes for the remainder of the report

Water is not separate from health. Water is not separate from disability. Water is not separate from mortality. The 19-year mortality gap documented in Section 2 is, in significant measure, the cumulative biological outcome of lifetime exposure to the water conditions documented in this section. The children born into Neskantaga's thirty-year advisory, the community members of Fort Chipewyan drinking from a river downstream of 1.5 trillion litres of process-affected tailings, the families in Pimicikamak and Kashechewan waiting for months of infrastructure repair after a

January 2026 failure — these populations are living inside the conditions that produce the disability profile this report is written to serve.

The jurisdictional fracture documented in Section 3 is the administrative design that makes the water crisis persistent. The federal government is responsible for on-reserve water. The provincial government regulates industrial water and opposes federal legislation that would establish enforceable First Nations water rights. Neither level of government is answerable to the person who turns on a tap in Neskantaga, or who draws drinking water from Lake Athabasca 250 kilometres downstream of Kearl, and who lives with the consequences. The housing documented in Section 4 is built over the same ground the contaminated water runs through. The tailings documented in Section 5 sit in the same watershed that produces the drinking water documented here. None of these sections describes a separate crisis. They describe the same crisis in four interlocking dimensions.

Section 7 will address the dual healthcare system — the federal Non-Insured Health Benefits program and the provincial Alberta Health Services system — through which Indigenous Albertans must navigate when the water-borne, housing-borne, and land-borne harms documented across these sections become the medical conditions that bring them into contact with the disability benefit system. The pattern is continuous. The documentation is sustained. The victim population is the same population across all the sections.

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